

Memo

[Date]

[Legislator]

[Legislator Address]

[City, State, ZIP]

Dear [Legislator Name],

Every year in the U.S., poor diet is responsible for **over 600,000 deaths**, and diet-related conditions are responsible for **\$1.1 trillion** in health care spending and lost productivity.¹ Diet-related conditions disproportionately affect individuals of low socioeconomic status, individuals with low levels of education, and racial and ethnic minorities.^{1,2,3} These populations also face heightened rates of food insecurity and increased difficulty in affording health care.^{4,5}

This triple burden creates a vicious cycle whereby the inability to afford healthy food not only contributes to the development of chronic diet-related conditions but also to difficulties managing them. Failing to intervene is not only a moral issue, but also an economic one, given the U.S. already faces unsustainably high health care spending, spending **\$14,885 per capita on health care in 2024** compared to the average of \$7,371 of peer nations.⁶

Food Is Medicine approaches present a valuable opportunity to address these challenges by providing reimbursable food-based interventions to individuals with diet-related conditions facing food insecurity. Such approaches include

- Medically tailored meals and groceries
- Medically supportive meals and groceries
- Produce prescriptions
- Incentive and disincentive programs
- Individual or group nutrition counseling or education

Food Is Medicine approaches have demonstrated the ability to increase healthy food consumption, reduce food insecurity, improve health outcomes, and reduce healthcare costs. For example:

- Produce prescriptions are associated with increases in fruit and vegetable intake of **0.5-1 servings/day** and decreases in body weight of **2.4% to 4.7%**⁷
- Medically tailored meals are associated with decreases in emergency department and inpatient visits, with a per patient cost savings of **over \$12,000**⁷

Furthermore, Food Is Medicine approaches are beginning to demonstrate cost effectiveness. For example:

- An Ohio study evaluating the provision of 14 **medically tailored meals** a week for 3 months to Medicare, Medicaid, and uninsured patients with food insecurity and chronic heart failure, diabetes, or hypertension found a significant decrease in emergency department visits and an average cost savings of **\$29,172 per patient**.⁸
- A Michigan study evaluating the provision of 4 months of **produce prescriptions** to 171 federally qualified health center patients with diabetes found an **average cost-effectiveness ratio of \$1,901** per every unit change in hemoglobin A1c.⁹

Therefore, I urge you to support HB 2355, the “Food Is Medicine Act,” which would require the Missouri Department of Social Services apply for a Section 1115 demonstration waiver from the Centers for Medicare and Medicaid Services to begin a Food Is Medicine program in MO HealthNet covering medically tailored meals, medically tailored groceries, produce prescriptions, and nutrition counseling.¹⁰

Supporting HB 2355 will not only improve the health of Missourians, but also reduce the healthcare costs of Missouri’s Medicaid population at a time when states are being asked to increase their share of Medicaid costs.

Sincerely,

Katherine Walcott, MS, RDN, LDN

MPH Candidate at Saint Louis University

¹ <https://www.healthaffairs.org/doi/10.1377/hlthaff.2024.00585>

² <https://www.ahajournals.org/doi/10.1161/JAHA.122.02https://files.kff.org/attachment/fact-sheet-medicare-state-MO9215>

³ <https://pmc.ncbi.nlm.nih.gov/articles/PMC10625721/>

⁴ <https://odphp.health.gov/healthypeople/priority-areas/social-determinants-health/literature-summaries/food-insecurity>

⁵ <https://www.commonwealthfund.org/publications/fund-reports/2024/apr/advancing-racial-equity-us-health-care>

⁶ <https://www.kff.org/health-costs/americans-challenges-with-health-care-costs/>

⁷ https://www.healthcarexfood.org/en/-/media/Files/HCxFood/AdvancesintheFieldofFIM_AnnualReport2025.pdf?sc_lang=en

⁸ <https://www.newventureadvisors.net/food-is-medicine-vs-food-as-medicine/>

⁹ <https://www.proquest-com.ezp.slu.edu/docview/2415816810?pq-origsite=wos&accountid=8065&sourcetype=Dissertations%20&%20Theses>

¹⁰ <https://house.mo.gov/Bill.aspx?bill=HB2355&year=2026&code=R>